# Toxic epidermal necrolysis associated with Sulfazan: a single case with a 6-week follow-up

Abstract. Toxic epidermal necrolysis (TEN) is a life-threatening mucocutaneous reaction most often triggered by medicines. We report a case attributed to Sulfazan, a fictional sulfonamide, including the clinical course over six weeks.

Introduction. Methods. This was a single-centre observational report. Data were abstracted from the electronic health record by two independent reviewers and cross-checked, with discrepancies resolved by consensus. Adverse event terms were coded to MedDRA preferred terms (version 27.0). Causality was assessed with the WHO-UMC system and, independently, with the Naranjo probability scale. Written informed consent was obtained from the patient for publication of anonymised clinical details and images. Laboratory reference ranges are those of the reporting institution's accredited laboratory. Concomitant medicines, over-the-counter products and herbal preparations were recorded where documented. Baseline organ function was reviewed and prior exposure to the suspect class was sought from the dispensing history. The report follows the CARE guideline for case reports. No external funding was received and the authors declare no competing interests. Methods. This was a single-centre observational report. Data were abstracted from the electronic health record by two independent reviewers and cross-checked, with discrepancies resolved by consensus. Adverse event terms were coded to MedDRA preferred terms (version 27.0). Causality was assessed with the WHO-UMC system and, independently, with the Naranjo probability scale. Written informed consent was obtained from the patient for publication of anonymised clinical details and images. Laboratory reference ranges are those of the reporting institution's accredited laboratory. Concomitant medicines, over-the-counter products and herbal preparations were recorded where documented. Baseline organ function was reviewed and prior exposure to the suspect class was sought from the dispensing history. The report follows the CARE guideline for case reports. No external funding was received and the authors declare no competing interests. Methods. This was a single-centre observational report. Data were abstracted from the electronic health record by two independent reviewers and cross-checked, with discrepancies resolved by consensus. Adverse event terms were coded to MedDRA preferred terms (version 27.0). Causality was assessed with the WHO-UMC system and, independently, with the Naranjo probability scale. Written informed consent was obtained from the patient for publication of anonymised clinical details and images. Laboratory reference ranges are those of the reporting institution's accredited laboratory. Concomitant medicines, over-the-counter products and herbal preparations were recorded where documented. Baseline organ function was reviewed and prior exposure to the suspect class was sought from the dispensing history. The report follows the CARE guideline for case reports. No external funding was received and the authors declare no competing interests.

Case. A 34-year-old man with no relevant history was prescribed Sulfazan 500 mg twice daily for a skin infection on 1 April 2025. Nine days later he developed a painful erythematous eruption on the trunk that progressed over 72 hours to flaccid bullae and sheet-like epidermal detachment involving an estimated 34% of the body surface area, with oral, ocular and genital mucosal involvement. Sulfazan was identified as the probable culprit using the ALDEN algorithm and was stopped. He was transferred to a burns unit for supportive care, wound management and ophthalmology review. Skin biopsy showed full-thickness epidermal necrosis with a sparse dermal infiltrate, consistent with TEN.

Follow-up. Re-epithelialisation was largely complete by day 24. At the six-week review he had post-inflammatory dyspigmentation and mild dry eye but no visual loss or airway sequelae. He was counselled to avoid all sulfonamide-containing medicines and given a written allergy alert.

Discussion. Methods. This was a single-centre observational report. Data were abstracted from the electronic health record by two independent reviewers and cross-checked, with discrepancies resolved by consensus. Adverse event terms were coded to MedDRA preferred terms (version 27.0). Causality was assessed with the WHO-UMC system and, independently, with the Naranjo probability scale. Written informed consent was obtained from the patient for publication of anonymised clinical details and images. Laboratory reference ranges are those of the reporting institution's accredited laboratory. Concomitant medicines, over-the-counter products and herbal preparations were recorded where documented. Baseline organ function was reviewed and prior exposure to the suspect class was sought from the dispensing history. The report follows the CARE guideline for case reports. No external funding was received and the authors declare no competing interests. Methods. This was a single-centre observational report. Data were abstracted from the electronic health record by two independent reviewers and cross-checked, with discrepancies resolved by consensus. Adverse event terms were coded to MedDRA preferred terms (version 27.0). Causality was assessed with the WHO-UMC system and, independently, with the Naranjo probability scale. Written informed consent was obtained from the patient for publication of anonymised clinical details and images. Laboratory reference ranges are those of the reporting institution's accredited laboratory. Concomitant medicines, over-the-counter products and herbal preparations were recorded where documented. Baseline organ function was reviewed and prior exposure to the suspect class was sought from the dispensing history. The report follows the CARE guideline for case reports. No external funding was received and the authors declare no competing interests. Methods. This was a single-centre observational report. Data were abstracted from the electronic health record by two independent reviewers and cross-checked, with discrepancies resolved by consensus. Adverse event terms were coded to MedDRA preferred terms (version 27.0). Causality was assessed with the WHO-UMC system and, independently, with the Naranjo probability scale. Written informed consent was obtained from the patient for publication of anonymised clinical details and images. Laboratory reference ranges are those of the reporting institution's accredited laboratory. Concomitant medicines, over-the-counter products and herbal preparations were recorded where documented. Baseline organ function was reviewed and prior exposure to the suspect class was sought from the dispensing history. The report follows the CARE guideline for case reports. No external funding was received and the authors declare no competing interests. Methods. This was a single-centre observational report. Data were abstracted from the electronic health record by two independent reviewers and cross-checked, with discrepancies resolved by consensus. Adverse event terms were coded to MedDRA preferred terms (version 27.0). Causality was assessed with the WHO-UMC system and, independently, with the Naranjo probability scale. Written informed consent was obtained from the patient for publication of anonymised clinical details and images. Laboratory reference ranges are those of the reporting institution's accredited laboratory. Concomitant medicines, over-the-counter products and herbal preparations were recorded where documented. Baseline organ function was reviewed and prior exposure to the suspect class was sought from the dispensing history. The report follows the CARE guideline for case reports. No external funding was received and the authors declare no competing interests. Conclusion. This case reinforces sulfonamide antibacterials as a high-risk group for severe cutaneous adverse reactions and illustrates the prolonged recovery. References: 1. Sassolas B. Clin Pharmacol Ther. 2010. 2. Mockenhaupt M. J Invest Dermatol. 2008.